

One workflow. A measurable next step.

Paid readiness and design-partner evaluation

Synthetic data only. No clinical deployment included.

USD 25,000

PROPOSED STANDARD FEE

8–12 weeks

AFTER SCOPE & READINESS

1 site / 1 workflow

REMOTE, ENGLISH-LANGUAGE

For clinics, medical centres and relevant technology partners: evaluate workflow fit, evidence traceability and implementation feasibility using a bounded HADP build and an agreed synthetic case set. No live integration is assumed. [1–3]

What is delivered

01 Workflow and evaluation charter

Define the problem, users, baseline tasks, permitted inputs, success criteria, exclusions and decision-makers.

02 Synthetic data and mapping pack

Agree fields, units, dates and provenance requirements. Record gaps and integration dependencies without receiving patient records.

03 Bounded demonstration and review

Review only functions supported for the agreed build. Label mock-ups and future functionality; log test results, limitations and reviewer feedback.

04 Decision and next-phase pack

Deliver the evidence readout, gap register, implementation estimate and a scoped next-step proposal: proceed, revise or stop.

An agreed sequence, not an automatic deployment

Scope and authorise → prepare synthetic cases → evaluate and record gaps → joint readout. A 12-week illustration allocates weeks 1–2, 3–4, 5–8 and 9–12 respectively. An 8-week schedule requires agreement on scope, staffing and readiness.

Evidence to measure

Traceability • task effort • physician-control records • reviewer usability • delivery feasibility. Baseline tasks, targets, review windows and acceptance evidence are agreed in the signed statement of work (SOW).

The paid result is a decision-quality evaluation package.

It is not unrestricted platform access, diagnostic validation, a treatment-outcome guarantee or permission for clinical use. A documented negative finding can still be a valid evaluation result.

Proposed scope and commercial structure. Contracting, delivery capacity and evaluation authority must be confirmed before work starts.

Clear payment. Defined delivery.

Standard fee: USD 25,000 — proposed milestone payments

50%	30%	20%
USD 12,500	USD 7,500	USD 5,000
Signed SOW and project start	Accepted workflow / configuration milestone	Final report and agreed handover acceptance

Acceptance is checked against the agreed deliverables, not against clinical benefit. The SOW defines review periods, payment due dates and any remediation process.

Alternative entry points — indicative proposals

Engagement	Proposed fee
Two-week readiness assessment	USD 7,500
Selected founding-partner evaluation	USD 20,000–25,000
Expanded evaluation / broader scope	USD 45,000–60,000
Multi-site / enterprise evaluation	From USD 75,000

Readiness: 100% at signing. Its USD 7,500 fee may be credited to an agreed pilot signed within 30 days; the credit is written into the SOW. Founding-partner terms require a defined scope, structured feedback and a next-phase decision process.

East Asia and Singapore: same core, scoped local work

The USD 25,000 base assumes one site, one workflow and remote English-language delivery. Hong Kong, Singapore, Japan, South Korea and mainland China may require different local work; location alone does not set the final fee.

Language localisation, additional integrations, local infrastructure, external specialist reviews and on-site work require a separate estimate. Pre-approved travel and accommodation are billed at cost. Taxes, currency and third-party charges are specified separately; no country-specific compliance is asserted.

Responsibilities and the signed SOW

VitaBahn provides scientific/technical contacts, an authorised evaluation environment, supported reviews and the evidence readout. The partner provides a workflow sponsor, reviewers, a budget owner, non-patient field specifications and review time.

The SOW names the contracting parties and signatories, scope, outputs, fees, milestones, acceptance criteria, confidentiality, IP/data rights, change control and exit terms. Later implementation and annual licensing require separate qualification and agreement; there is no automatic conversion or exclusivity.

Boundary: no real patient data, including de-identified or pseudonymised records. Any later patient-data or clinical phase needs separate technical, privacy/security, clinical/regulatory and contractual approvals. [3]

Next step: a 30-minute scoping session.

Dr. Motaz Shieban

[1–3] Source basis: existing Pilot Partner Brief v1.0. Prices and terms are proposals from the Hong Kong planning discussion, not verified market rates, incurred costs or an executed agreement.